AZUKON MR

(Gliclazide Modified Release Tablets 30mg)

NAME AND STRENGTH OF ACTIVE SUBSTANCE(S)

AZUKON MR

Each uncoated modified release tablet contains

Gliclazide. ………….. 30mg

PRODUCT DESCRIPTION:

White to off-white coloured, capsule shaped uncoated tablets plain on both sides.

DOSAGE FORM:

Uncoated Tablets

PHARMACODYNAMICS:

Gliclazide stimulates the secretion of insulin from functioning pancreatic islet β-cells. In addition to this pancreatic action, it has been demonstrated that Gliclazide administration may improve the metabolic utilization of glucose at a peripheral level.

PHARMACOKINETICS:

Absorption

Gliclazide PR shows linear pharmacokinetics over the 15 to 120mg dose range in patients with type 2 diabetes mellitus. The intra-individual variability is low, at 16%. Cmax is reached at about 6 hours after administration (tmax). Fasting Cmax in 16 healthy volunteers given a single 30 mg dose of Gliclazide PR was 0.74 mg/L at a tmax of 7 hours, and the area under the plasma concentration- time curve (AUC) was 16.2 mg/L .h. The mean absolute bioavailability of Gliclazide was 97% (range 79 to 110%) after administration of a single oral dose of Gliclazide PR 30mg to 16 healthy volunteers.

Distribution

The volume of distribution of Gliclazide in healthy and diabetic subjects was 15.9 to 17 or 20 to 40% of body weight. Gliclazide is highly bound to albumin (95%).

Metabolism

Gliclazide is oxidized to produce either hydroxylated metabolites or N-oxygenated compounds, and the corresponding alcohol and carboxylic acid. Gliclazide is extensively metabolized, less than 20% of the dose being excreted unchanged in the urine. However in plasma, gliclazide represents over 90% of all drug related material, the most important metabolite being present at the extent of 1%. This metabolite has not demonstrated any hypoglycemic action but claims have been made that it possess effects on platelet aggregation and micro thrombi formation. The other metabolites seen in urine are only present in trace quantity (less than 20 ng/ml) in plasma.

Excretion

Studies in man have reported that, 60 to 70% of the dose to be excreted in the urine and 10 to 20% in the faeces. Elimination in human faeces was slower than with other species. The extent of protein binding in man would hinder the glomerular filtration process, whilst gliclazide lipid solubility would enhance reabsorption from the tubules. The same value for plasma clearance (13.5 ml/min) was found in a group of elderly subjects (mean age 77 years) as in a group of younger subjects (mean age 26 years).

In patients with type 2 diabetes mellitus the apparent clearance of Gliclazide PR was 0.9 L/h, with an apparent volume of distribution of 19L. Plasma concentrations declined exponentially, with an elimination half-life (t1/2) of approximately 16 hours.

INDICATION:

Gliclazide is indicated in Therapy of maturity onset Diabetes Mellitus (noninsulin-dependent or Type II), where dietary management alone has been insufficient.

RECOMMENDED DOSAGE:

Adults: The modified release formulation allows convenient once daily dosing of 1-4 tabs/day recommended to be taken followed by a meal.

Elderly: In the elderly (>65 years) Gliclazide MR should be prescribed using the same dosing regimen recommended for patients <65 years. In patients with mild to moderate renal insufficiency, the same dosing regimen can be used as in patients with normal renal function with careful patient monitoring. Care should be exercised however, when prescribing sulphonylureas in the elderly due to a possible age-related increased risk of hypoglycaemia.

These data have been confirmed in clinical trials.

Children: Gliclazide, as with other sulphonylureas, is not indicated for the treatment of juvenile onset diabetes mellitus.

It is preferable to take Gliclazide MR at breakfast in order to avoid or minimize the possible onset of digestive disorders. Switching Gliclazide to Gliclazide MR should be done on a tablet for tablet basis eg, 1 tab with breakfast and 1 tab in the evening of Gliclazide has to be switched to 2 tabs of Gliclazide MR in the morning.

When a dose is forgotten, it is recommended not to take a double dose to compensate for the missed dose.

If the treatment is discontinued, there is a risk of an imbalance in diabetes (hyperglycemia). In all cases, strictly comply with the doctor's prescription.

Initial dose

The recommended starting dose is 30 mg daily.

If blood glucose is effectively controlled, this dose may be used for maintenance treatment.

If blood glucose is not adequately controlled, the dose may be increased to 60, 90 or 120 mg daily, in successive steps. The interval between each dose increment should be at least 1 month except in patients whose blood glucose has not reduced after two weeks of treatment. In such cases, the dose may be increased at the end of the second week of treatment.

The maximum recommended daily dose is 120 mg.

Switching from Gliclazide 80 mg tablets to Gliclazide 30 mg modified release tablets:

1 tablet of Gliclazide 80 mg is comparable to 1 tablet of Gliclazide 30 mg MR Tablets.

Consequently the switch can be performed provided a careful blood monitoring.

Switching from another oral antidiabetic agent to Gliclazide 30 mg MR Tablets.

Gliclazide 30 mg MR Tablets can be used to replace other oral antidiabetic agents.

The dosage and the half-life of the previous antidiabetic agent should be taken into account when switching to Gliclazide 30 mg MR Tablets.

A transitional period is not generally necessary. A starting dose of 30 mg should be used and this should be adjusted to suit the patient's blood glucose response, as described above.

When switching from a hypoglycaemic sulphonylurea with a prolonged half-life, a treatment free period of a few days may be necessary to avoid an additive effect of the two products, which might cause hypoglycaemia. The procedure described for initiating treatment should also be used when switching to treatment with Gliclazide 30 mg MR Tablets, i.e. a starting dose of 30 mg/day, followed by a stepwise increase in dose, depending on the metabolic response.

Combination treatment with other antidiabetic agents

Gliclazide 30 mg MR Tablets can be given in combination with biguanides, alpha glucosidase inhibitors or insulin.

In patients not adequately controlled with Gliclazide 30 mg MR Tablets, concomitant insulin therapy can be initiated under close medical supervision.

MODE OF ADMINISTRATION:

Oral, Should be taken with food (Take immediately before meals. Swallow whole, do not chew / crush).

CONTRAINDICATIONS:

Hypersensitivity to sulphonylureas and related substances. Not to be used for: type 1 diabetes, juvenile onset diabetes; diabetes complicated by ketosis or acidosis; diabetics undergoing surgery, after severe trauma or during infections; diabetic precoma and coma; severe renal or hepatic insufficiency; porphyria, hyperthyroidism, pregnancy and lactation.

WARNINGS AND PRECAUTIONS:

Hypoglycemia: As with other sulfonylureas, gliclazide is susceptible to cause episodes of hypoglycemia (decrease of blood sugar levels). In the case of sweating, intense hunger, trembling, pallor, visual disturbances, feeling of malaise, abnormal behavior, immediately eat sugar or something containing sugar and inform the doctor.

Hypoglycemia is favored by a strict or poorly balanced diet, by prolonged or strenuous exercise, by the intake of alcohol or during use of other hypoglycemic drugs.

In consequence, treatment with a hypoglycemic sulfonylurea requires: Regular Diet: It is important to eat regular meals, including breakfast, never to miss a meal, due to an increase in the risk of the onset of hypoglycemia caused by an inadequate diet or a sugar imbalance.

Precise Regularity in the Administration of Treatment: It is important to take treatment regularly, once a day, preferably at breakfast.

Every administration of gliclazide MR must be accompanied by a meal.

INTERACTIONS WITH OTHER MEDICAMENTS:

Care should be taken when using Gliclazide with medicines, which are known to alter the diabetic state or potentiate the medicine's action. The hypoglycaemic effect of Gliclazide may be potentiated by phenylbutazone, salicylates, sulphonamides, coumarin derivatives; monoamine oxidase inhibitors, beta-adrenergic blocking agents, tetracycline compounds, chloramphenicol, clofibrate, disopyramide, oral forms of miconazole and cimetidine. The hypoglycaemic action of Gliclazide may be diminished by corticosteroids, oral contraceptives, thiazide diuretics, phenothiazine derivatives, thyroid hormones and abuse of laxatives.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Pregnancy:

As pregnant women require tight glycemic control, insulin would be better choice.

Lactation:

Human data are not available about excretion of gliclazide in breast milk. Nursing mothers taking sulphonylureas including gliclazide should not breastfeed.

ADVERSE EFFECTS / UNDESIRABLE EFFECTS:

Hypoglycemia: Gliclazide, as other sulphonylureas, is capable of producing moderate to severe hypoglycaemia, particularly in the following conditions: In patients controlled by diet alone; in case of accidental overdose; when calorie or glucose intake is deficient; in patient with hepatic and/or renal impairment, however, in long-term clinical trials, patients with renal insufficiency have been treated satisfactorily, using Gliclazide at reduced doses. Dosage adjustments may be necessary, on the occurrence of mild symptoms of hypoglycemia (sweating, pallor, hunger pangs, tachycardia, sensation of malaise). Such findings should be treated with oral glucose and adjustments made in medicine dosage and/or meal patterns; on the occurrence of severe hypoglycaemic reactions (coma or neurological impairments, see overdosage), loss of control of blood glucose (hyperglycaemia). When a patient stabilized on any diabetic regimen is exposed to stress such as fever, trauma, infection or surgery, a loss of control may occur. At such times it may be necessary to progressively increase the dosage of Gliclazide and if it is insufficient to discontinue the treatment of Gliclazide and to administer insulin.

Abnormalities of hepatic function may occur during Gliclazide therapy. There are less frequent reports of hepatic failure, hepatitis and jaundice following treatment with Gliclazide.

Mild gastrointestinal disturbances including nausea, dyspepsia, diarrhea and constipation have been reported, but this type of adverse reaction can be avoided if Gliclazide is taken with a meal

Weight change: Most studies reported no significant overall change in weight. Skin reactions including rash, pruritus, erythema, bullous eruption, blood dyscrasia including anaemia, leucopenia, thrombocytopenia and granulocytopenia have been observed dur ing treatment with Gliclazide. Facial flushing may develop in patient receiving sulphonylureas.

Care should be exercised in patients with hepatic and/or renal impairment and a small starting dose should be used with careful patient monitoring. As with other sulphonylueas, hypoglycaemia will occur if the patient's dietary intake is reduced or if they are receiving a larger dose of Gliclazide than is required.

Effects on ability to drive and use machines

Patients should be informed that their concentration might be affected if their diabetes is not satisfactorily controlled, especially at the beginning of treatment.

Laboratory Test Abnormalities: Apart from 1 or 2 isolated cases of raised concentrations of AST and /or ALT no significant alteration of hepatic function has been reported. Blood urea nitrogen levels have been reported to be both increased and decreased. Similarly, both significant increases and significant decreases in eosinophiles have been documented as well as decreased red and white blood cell counts in different trials. Gliclazide was shown to have no effect on thyroid functions in 15 diabetic patients given 160mg for 6 months.

OVERDOSE AND TREATMENT:

Hypoglycaemic reactions should be treated by gastric lavage and correction of the hypoglycaemia by the administration of intravenous glucose. The patient's blood sugar should be continuously monitored until the effect of the drug has ceased. (This may take several days).

Hypoglycaemic reactions should alert the physician to the possibility of renal dysfunction.

STORAGE CONDITIONS:

Store below 30°C. Protected from moisture.

Keep out of reach of children.

PACKAGING AVAILABLE:

Azukon MR Tablets is packed in Alu-PVC blister of 10 tablets. Such 10 blister are packed into a carton.

Not all presentations may be available locally.

EXPIRY DATE:

24 months from the date of manufacturing

DATE OF REVISION OF PACKAGE INSERT

25 August, 2021

NAME AND ADDRESS OF MANUFACTURER

torrent PHARMA

Manufactured by:
TORRENT PHARMACEUTICALS LTD.
Indrad-382 721, Dist. Mehsana, INDIA.

PRODUCT NAME	: Azukon MR	**COUNTRY :** Malaysia	**LOCATION :** Indrad	**Supersedes A/W No.:**			
ITEM / PACK	: Insert	**NO. OF COLORS:** 1	**REMARK :**				
DESIGN STYLE	: Front/Back	**PANTONE SHADE NOS.:**	**SUBSTRATE :**				
CODE	: xxxxxxxxx-5253		**Activities**	**Department**	**Name**	**Signature**	**Date**
DIMENSIONS (MM)	: 150 x 220		Prepared By	**Pkg.Dev**			
ART WORK SIZE	: S/S		Reviewed By	**Pkg.Dev**			
DATE	: 25-08-2021	Black	Approved By	**Quality**			